



Occupational Therapy: Mental Health Procedure

1. Applicability

Site	Operational Area	Applicable to
Armadale Mental Health Service	Inpatient Mental Health	Occupational Therapy

2. Purpose

To ensure consistent occupational therapy assessment, intervention and discharge planning processes for consumers admitted to the mental health wards at Armadale Health Service.

3. Procedure

3.1 Assessment

- Prior to assessment, the occupational therapist will review the iSOFT clinical handover and patient's health care records including any previous occupational therapy assessments.
- Complete initial Occupational Therapy (OT) screen to determine if urgent OT input is required.
- Commence comprehensive Occupational Therapy (OT) initial evaluation as indicated.
- A comprehensive initial evaluation does not need to be completed if it is available in the health care record and completed within the past twelve months. If significant information has changed an updated evaluation is required.
- If patients cannot provide an accurate history due to cognitive or physical impairment or decline in mental state, consent from the patient will be obtained to contact the next of kin for collateral information if appropriate.
- The occupational therapist will identify clinical need for further comprehensive assessment, which may include:
 - Suitability for the ward group program
 - Equipment prescription Cognitive screening and assessment
 - Bariatric risk assessment ([AKMR111.2.1](#))
 - Falls risk assessment
 - Interest checklist
 - Sensory assessment
 - Functional assessment- ward and community based
 - Home assessment (AKMR 61B)
 - Pressure care
 - Medication skills assessment
- When applicable, the occupational therapist reviews the Skin and Pressure Injury Assessment (AKMR121.9) and the Falls Risk Assessment and Management Plan (FRAMP – AKMR24) and updates accordingly.

Before referencing this procedural document please ensure that you have the latest version from the [Policy Library](#) information hub page.

- Where applicable, the occupational therapist will conduct community based functional assessments to assess IADL capacity
- When a National Disability Insurance Scheme (NDIS) access request or discharge assessment is to be submitted, the occupational therapist will complete a functional capacity assessment to submit as evidence for the patient's application.
- Where a patient's functional home assessment is indicated, the occupational therapist will consult with the multidisciplinary team to ensure the patient is appropriate to attend a home assessment during their inpatient admission.
- When it is identified that a home assessment is required prior to discharge, the assessment is to be booked as soon as possible to allow adequate time for equipment provision and installations/modifications in the home. Patients will be provided with equipment and home modifications which is deemed 'basic and essential' for discharge in accordance with [Allied Health Provision of Aids/Equipment and Home Modifications](#)
- The assessment data is analysed, interpreted, and summarised to determine the patient's current functional status and develop a relevant, client centered intervention plan; this is recorded in the Treatment Support and Discharge Plan (TSDP) by the occupational therapist.

3.2 Intervention

- In collaboration with the consumer the occupational therapy team will implement actions, goals, and documents in the TSDP. This may include a combination of individual sessions, groups, further assessments, or referrals. The TSDP will be evaluated regularly during the consumers admission.
- Occupational therapy intervention will focus on motivation, leisure, habits/routines, valued activities/roles, prevocational rehabilitation, life skills, functional cognition, physical activity, communication/interaction, self-management, emotional regulation, and environment.
- The occupational therapist will use strategies depending on the patient's needs or requirements to support and facilitate their engagement in the intervention process and achievement of their goals including therapeutic use of self, therapeutic use of occupations/activities, motivational interviewing, Cognitive Behavioural Therapy (CBT), Dialectical Behavioural Therapy (DBT), validation and counselling, goal setting, behavioural activation, sensory modulation, grading, creative activities, structuring, coaching and physical support.
- Individual sessions provided by the occupational therapist will be personalised to the patients' needs and may include the following:
 - Assisting patients to build personal aspirations through identifying strengths, and transferable skills from past activities.
 - Enabling patients to establish a sense of purpose and/or direction.
 - Education regarding mental health diagnoses or symptoms and the impact on daily activities and function.
 - Providing education and support in developing positive coping skills, symptom management and distress tolerance.
 - Re/establishing balanced, productive, and meaningful daily routines and roles which provide structure to self-care, leisure, and productivity activities.
 - Provide opportunities to practice and maintain self-care, leisure and productive skills prioritised by the individual in environments that best suit them.
 - Education regarding cognitive strategies to maximize independence in ADL's.

- Education regarding falls prevention strategies
 - Provision of equipment to promote safe and independence engagement in ADLs
 - Pressure care management including provision of support surfaces and education
 - Support the patient in developing or maintaining a satisfying personal or social identity through community engagement or access to culturally appropriate social activities and support groups.
 - The occupational therapist will consult and liaise with community services and supports, advocate for and complete reports necessary to facilitate a safe discharge, including; Exploring meaningful activity and identifying community resources available to meet these needs, which may include referral and introduction to local community clubs, befriending services, and leisure resources.
 - Carer education and support in relation to maximising the patient's activity and functioning in their home environment.
 - Groups are activity and/or skills based and aim to increase experiences of enjoyment and satisfaction through meaningful activity and/or to develop interpersonal, social, and coping skills.
- For patients at risk or who have a pressure injury, refer to intervention requirements in [Occupational Therapy Pressure Injury Prevention and Management](#) procedure.
 - Interventions will be regularly re-evaluated and modified to reflect changes in the patient's functional status, mental state, goals, and response to intervention.
 - Where specialised or individualised equipment is prescribed for ward use, handover and education will be provided to nursing staff, recorded on iSOFT, TSDP and documented in the patients' medical record.
 - The patient, family and carers will be provided with all necessary information regarding equipment, home modifications and other education required.

3.3 Discharge Planning

- Where equipment is essential for discharge this will be supplied prior to or on the day of discharge.
- Where applicable, complete discharge summary assessments for NDIS participants.
- All relevant documentation will be completed before discharge including, ISOBAR handover, NaCS and when required OT and Physiotherapy Transfer Form ([AKMR62.5](#)) to facilities and services.

4. Compliance monitoring

Compliance against this procedure will be evaluated and monitored via file audits of the occupational therapy documentation.

- Guidelines

5. Supporting information

The following documents support the implementation of this procedure:

- [Occupational Therapy Mental Health Initial Evaluation ALL-GUI-0323-18](#)
- [Occupational Therapy Home Assessments ALL-PRO-0482-17](#)
- [Occupational Therapy Pressure Injury Prevention and Management AKG-PRO-0320-17](#)

- [Allied Health Provision of Aids/Equipment and Home Modifications AKG-PRO-0028-17](#)
- [Documentation Standards Clinical AKG-PRO-0134-11](#)
- East Metropolitan Health Service [Patient Identification and Procedure Matching EMHS: 21](#)

6. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.10 Communication that supports effective partnerships
- Standard 5 – Action 5.7 Planning for comprehensive care
- Standard 5 – Action 5.10 Screening of risk
- Standard 5 – Action 5.13 Developing the comprehensive care
- Standard 6 – Action 6.11 Documentation of information

7. References

1. Occupational Therapy Australia Limited (2014). Occupational Therapy Practice in Mental Health Section 4: Occupational Therapy Guide to Rehabilitation Interventions. [online] Rose Park, South Australia, pp.1 - 17. Available at: <https://www.otaus.com.au/sitebuilder/divisions/knowledge/asset/files/69/interventiondoccountilapproved4-15.pdf> [Accessed 4 Jun. 2019].
2. Wheeler, K. (2015). Occupational Therapy Assessments and Interventions in Mental Health: Campus and Neighbourhood. [online] Derbyshire Healthcare NHS Foundation Trust, pp.1 - 13. Available at: <https://docplayer.net/28496564-Occupational-therapy-assessments-and-interventions-in-mental-health-campus-and-neighbourhood.html> [Accessed 4 Jun. 2019].

8. Document contact

Enquiries relating to this procedure may be directed to:

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9. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	30/10/2024	30/10/2027	3.1 Assessment – Additional of statement relating to completing an initial OT screen. Removal of “within three business days from point of referral and/or admission”. Re-wording from “a new form is to be completed” to “an updated evaluation is required”. Additional of “suitability for the ward program”, “equipment prescription”, “falls

			risk assessment and huddle”, “medication skills assessment” and identifying “ward and community based” and functional assessments, added to comprehensive Ax section. Removal of general OT Assessments from list (Home Ax form, BMI, kitchen task Ax and FIM) Additional of statement relating to community based functional assessments Additional of statement relating to NDIS assessments Wording: Additional of “discharge assessment” included in item covering NDIS applications 3.2 Intervention: Inclusion of tem “TSDP” Inclusion of “CBT and DBT” Addition of statements relating to cognitive strategies, falls prevention, equipment provision and pressure care management. Addition of statement relating to: consultation and liaison with community services and supports Addition of statement relating to: discharge summaries for NDIS participants and the term ‘ISOBAR handover”
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10. Authorisation

This procedure has been authorised and issued by:

Authorised by	Karen Elliott: Service Director Mental Health
Authorisation date	29/10/2024
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